

Minnesota Medicaid Fraud: Red Flag Library (Source Typology)

Note on terminology: This library describes Minnesota's own, already-charged or adjudicated enforcement cases — "red flag" is used here in its original sense, as the label these patterns carry in Minnesota's audit and enforcement record. When this typology is adapted for Washington, where no comparable enforcement record exists, this project calls the adapted indicators "review-priority indicators" rather than red flags, to avoid implying that identifying a pattern in Washington is equivalent to Minnesota's charged conduct. See [WA_Transferability_Memo.md](#) and the manuscript for that reframing.

Based on the 2024–2026 enforcement wave, these 20 patterns (RF-01 to RF-20) represent the most common programmatic, administrative, and billing anomalies observed in Minnesota's charged and adjudicated HCBS and residential-provider fraud cases.

Category 1: Program & Provider Enrollment Anomalies

- **RF-01: Rapid Provider Enrollment Growth:** A sudden, exponential increase in the number of new providers registering for a specific service line (e.g., HSS, EIDBI) within a 12–24 month period.
- **RF-02: Spending Growth Disconnected from Beneficiary Population:** Program spending increases at a rate (e.g., HSS growing from \$21M to \$104M in three years) that mathematically cannot be explained by the natural incidence rate of the qualifying condition.
- **RF-03: Extreme Billing Concentration:** A small percentage of providers capturing a massive share of total program payments (e.g., OLA found 10% of HCBS providers captured 70% of payments).
- **RF-04: Provider "Fraud Tourism":** Out-of-state individuals with no local ties registering businesses to provide high-touch, in-person community services (e.g., Pennsylvania residents running MN HSS agencies).
- **RF-05: Incomplete Administrative Data:** Providers failing to supply complete background, credentialing, or ownership data during enrollment or revalidation (the primary cause of 2,491 disenrollments in MN).
- **RF-06: Excluded Party Involvement:** Individuals on the federal OIG exclusion list secretly managing or operating a provider entity registered under a "clean" straw owner's name.
- **RF-07: Concealed Ownership of Housing:** Medicaid providers secretly owning the residential properties where their clients live (violating conflict-of-interest rules) to control the clients and their billing data.
- **RF-08: Undisclosed Related-Party Networks:** Multiple provider agencies sharing the same registered agents, managers, or addresses without disclosing the affiliation.
- **RF-09: Phantom or Mailbox Addresses:** Providers billing millions for community services while operating out of a P.O. box, a residential apartment, or a virtual office suite with no physical infrastructure.

Category 2: Billing & Claims Anomalies

- **RF-10: Impossible Hours (Overlapping/Excessive):** Providers billing for more than 24 hours in a single day, or billing for simultaneous services provided to different clients in different locations.
- **RF-11: Maximum Unit Billing / Inflated Units:** Providers routinely billing the absolute maximum allowable daily units for every client, regardless of the client's actual acuity or care plan.
- **RF-12: Impossible Locations (Out of State):** Billing for in-person daily care while the provider, the recipient, or both are out of the state or out of the country.
- **RF-13: Billing for Deceased Recipients:** Submitting daily service claims for days or weeks after a recipient's documented date of death.
- **RF-14: Cloned or Fabricated Documentation:** Submitting identical, copy-pasted progress notes (same start/end times, same text) for every client every day, or backdating/manufacturing notes only when audited.
- **RF-15: Stolen Professional Credentials:** Using the NPI or license number of a legitimate medical professional (without their knowledge) to bill for services actually performed by unqualified staff.

Category 3: Oversight & Systemic Failures

- **RF-16: Ignored Frontline Referrals:** State agencies failing to act on detailed, written fraud referrals submitted by county social workers or managed care SIUs.
- **RF-17: Political or Social Pressure Chilling Oversight:** Regulators overriding frontline investigators or resuming suspended payments due to pressure from lobbyists, legislators, or claims of community targeting.
- **RF-18: Chronic Oversight Capacity Shortages:** Regulatory units operating with insufficient investigative staff, no case management systems, or relying solely on reactive complaints rather than proactive data mining.
- **RF-19: Program Design Vulnerability:** Implementing new, high-dollar waiver services with "low barriers to entry and minimal records requirements for reimbursement," prioritizing rapid rollout over financial controls.
- **RF-20: Unresolved Prior Audit Findings:** State agencies repeatedly failing to correct structural vulnerabilities identified years earlier by state or federal auditors (e.g., MN DHS failing to implement 2017 OLA HCBS financial oversight recommendations).